

1. Administrative & Study Identification

Full Study Title: A Study of Coformulated Favezelimab/ Pembrolizumab (MK-4280A) Versus Standard of Care in Subjects With Previously Treated Metastatic PD-L1 Positive Colorectal Cancer (MK-4280A-007)-China Extension Study **Official Title:** A Phase 3 Study of MK-4280A (Coformulated Favezelimab [MK-4280] Plus Pembrolizumab [MK-3475]) Versus Standard of Care in Previously Treated Metastatic PD-L1 Positive Colorectal Cancer (KEYFORM-007) **Short Title/Acronym:** MK-4280A-007 China Extension **Protocol Number:** MK-4280A-007 **NCT Number:** NCT05600309 **Posting ID:** 865249763829750483 **Sponsor/Company:** Merck **Investigational Product:** Coformulated favezelimab/pembrolizumab (MK-4280A), Regorafenib (Stivarga), TAS-102 **Phase of Development:** Phase 3 **Medical Monitor:** Medical Director, Merck

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To assess the safety and efficacy of coformulated favezelimab/pembrolizumab (MK-4280A) versus standard of care (regorafenib and TAS-102) with respect to overall survival (OS). The primary study hypothesis is that coformulated favezelimab/pembrolizumab (MK-4280A) is superior to standard of care with respect to overall survival. **Secondary Objectives:** To evaluate Progression-Free Survival (PFS), Objective Response Rate (ORR), and Duration of Response (DOR). *(Note: As of Amendment 3 of the supplemental statistical analysis plan, patient-reported outcomes will no longer be secondary outcome measures of the study.)*

2.2 Background & Rationale To address the need for improved therapeutic strategies in metastatic colorectal cancer (mCRC), a condition where traditional modalities often have limited efficacy. Targeting the PD-1/PD-L1 axis alongside LAG-3 (lymphocyte activation gene 3) provides a next-generation immune checkpoint blockade approach. This China extension study evaluates the clinical benefits of dual immune checkpoint blockade in adult Chinese patients with metastatic PD-L1 positive colorectal cancer who have already progressed on standard therapies.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active Comparator (Standard of Care: regorafenib or TAS-102)

Blinding: Open-label (None) **Randomization:** Randomized, Parallel Assignment

3.2 Planned Enrollment & Duration Trial Status: COMPLETED **Target Enrollment:** Approximately 94 Chinese participants (includes participants previously enrolled in China in the global study for MK-4280A-007 plus those enrolled during the China extension enrollment period). **Number of Sites:** Multicenter (China extension sites) **Estimated Study Start:** 14-Jun-2022

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Has a histologically confirmed colorectal adenocarcinoma that is metastatic and unresectable. 2. Has measurable disease per RECIST 1.1 as assessed by the local site investigator. 3. Has been previously treated for the disease and radiographically progressed on or after or could not tolerate standard treatment. 4. Submits an archival (≤ 5 years) or newly obtained tumor tissue sample or newly obtained tumor tissue sample that has not been previously irradiated. 5. Has an Eastern Cooperative Oncology Group Performance Score (ECOG PS) of 0 to 1 within 10 days prior to first dose of study intervention. 6. Has a life expectancy of at least 3 months, based on investigator assessment. 7. Has the ability to swallow and retain oral medication and does not have any clinically significant gastrointestinal abnormalities that might alter absorption. 8. Has adequate organ function.

4.2 Exclusion Criteria 1. Has previously been found to have deficient mismatch repair/microsatellite instability-high (dMMR/MSI-H) tumor status. 2. Has known active central nervous system (CNS) metastases and/or carcinomatous meningitis or leptomeningeal disease. 3. Has a history of acute or chronic pancreatitis. 4. Has neuromuscular disorders associated with an elevated creatine kinase (e.g., inflammatory myopathies, muscular dystrophy, amyotrophic lateral sclerosis, spinal muscular atrophy). 5. Has clinically significant cardiovascular disease within 12 months from first dose of study intervention (e.g., NYHA Class III/IV heart failure, unstable angina, myocardial infarction, CVA, or cardiac arrhythmia with hemodynamic instability). 6. Has urine protein greater than or equal to 1g/24h. 7. Is a woman of childbearing potential who has a positive urine/serum pregnancy test within 24/72 hours prior to the first dose of study intervention. 8. Has received prior therapy with an anti-PD-1, anti-PD-L1, anti-PD-L2, anti-LAG-3 antibody, or with a TKI (e.g., lenvatinib) other than RAF inhibitors, or an agent directed to another stimulatory/co-inhibitory T-cell receptor. 9. Has previously received regorafenib or TAS-102. 10. Has received prior systemic anticancer therapy (including investigational agents) within 28 days before randomization. 11. Has received prior radiotherapy within 2 weeks of start of study intervention (must be recovered, not require steroids, no radiation)

clinical mortality assessment and continuous survival follow-up tracking.

7.2 Secondary Endpoints * Progression-Free Survival (PFS):

According to Response Evaluation Criteria in Solid Tumors Version 1.1 (RECIST 1.1) as Assessed by Blinded Independent Central Review (BICR). * **Objective Response Rate (ORR):** Per RECIST 1.1 as Assessed by BICR. * **Duration of Response (DOR):** Per RECIST 1.1 as Assessed by BICR. (*Patient reported outcomes are excluded as per Amendment 3*).

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous clinical and laboratory assessments during onsite visits and remote tracking (e.g., evaluating for immune-mediated events like pneumonitis). **Serious Adverse Events (SAEs):** Standard reporting within 24 hours of site awareness. **Data Monitoring Committee (DMC):** Independent data monitoring committee for periodic safety analyses.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for primary survival outcomes. Safety Set for all AE/SAE evaluations. **Sample Size Justification:** Target enrollment of 94 Chinese participants to appropriately power the China cohort extension in supplementing the global MK-4280A-007 data. **Handling of Missing Data:** Standard oncologic statistical censoring applied for OS and PFS survival endpoints per RECIST 1.1 rules.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Scheduled onsite and remote clinical monitoring visits to ensure RECIST 1.1 adherence and exact safety data collection. **Audit Trail:** eCRF locking and Blinded Independent Central Review (BICR) tracking mechanisms for PFS confirmation.

11. Study Identifiers & Governance

Primary ID: {{MK-4280A-007}} **Posting ID:** {{865249763829750483}}
Registry Number: {{NCT05600309}} **Sponsor/Lead Organization:** {{Merck}} **Study Title:** {{A Study of Coformulated Favezelimab/Pembrolizumab (MK-4280A) Versus Standard of Care in Subjects With Previously Treated Metastatic PD-L1 Positive Colorectal Cancer (MK-4280A-007)-China Extension Study}} **Official Regulatory Title:** {{A Phase 3 Study of MK-4280A (Coformulated Favezelimab [MK-4280]

Plus Pembrolizumab [MK-3475]) Versus Standard of Care in Previously Treated Metastatic PD-L1 Positive Colorectal Cancer (KEYFORM-007))

12. Clinical Framework (Colorectal Cancer Specific)

Disease Area: {{Colorectal Cancer}} **Primary Condition:** {{Metastatic Colorectal Carcinoma}} **Preferred UMLS Name:** {{Metastatic colorectal carcinoma}} **MeSH Code:** {{D015179}} **SNOMED ID:** {{363406005}} **Diagnosis Threshold:** {{PD-L1 Positive, dMMR/MSI-H Excluded (MSS/pMMR)}} **Medical Methodology:** {{Dual Immune Checkpoint Blockade (Anti-LAG-3/Anti-PD-1) vs. SOC}} **Optimization Target:** {{Overall Survival (OS) improvement}} **Drug Trade Name (SOC):** {{Stivarga (regorafenib)}} **Drug ATC Code (SOC):** {{L01EX05 (regorafenib)}}

13. Study Procedures & Methodology

Management Pathway: {{Standard oncologic clinical pathway for mCRC}} **Education Protocol (HCP):** {{RECIST 1.1 evaluation & immune-mediated AE management training}} **Education Protocol (Patient):** {{IV infusion scheduling and oral SOC compliance diary}} **Quality Audit Mechanism:** {{Blinded Independent Central Review (BICR) for tumor response}}

14. Observation & Follow-Up Schedule

Total Duration: {{Until disease progression or unacceptable toxicity}} **Onsite Visit Cadence:** {{Every 3-4 weeks depending on randomized arm schedule}} **Remote Monitoring Frequency:** {{Continuous safety and survival tracking}} **Checklist Review Interval:** {{Tumor imaging evaluation every 9 weeks}}

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				